



ICD-O-3

PATH: Carcinoma w/osteoclast-like
Giant Cells 8635/3

CSCF: Carcinoma, Urothelial NOS 8120/3

Gender: Male

Path Site: Bladder, posterior and lateral wall
C67.8

Race: White

CSCF Site: Bladder, posterior wall C67.4

Pathology Report:

gwd 12/24/12

Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

A. Left pelvic lymph node; lymphadenectomy:

- Eight lymph nodes, no tumor (0/8).

B. Urinary bladder, prostate, left and right vas deferens;
cystoprostatectomy:

- Bladder with invasive osteoclast-like giant cells rich poorly differentiated urothelial carcinoma, invading thru the muscularis propria and into the perivesical soft tissue, see pathologic parameters and comment.
- Tumor shows multifocal lymphovascular invasion and spread within the bladder wall to perivesical soft tissues.
- Posterior bladder and right ureter margins, positive for tumor.
- Prostate with incidental prostatic adenocarcinoma, see pathologic parameters.

C. Right pelvic lymph nodes; lymphadenectomy:

- Soft tissue deposits of metastatic osteoclasts-like giant cell rich urothelial carcinoma, see comment.

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive poorly differentiated osteoclast-like giant cells rich urothelial carcinoma, see comment.
2. Grade of tumor: High grade.
3. Depth of invasion: Extravesicular soft tissue (macroscopic).
4. Tumor distribution: Large mass (4.0 cm) at inferior posterior and right lateral wall, and with multiple small tumor foci in bladder wall at anterior, posterior, right lateral and trigonal walls; additional free-floating/detached tumor fragments that measured 11 cm and 3.2 cm.
5. Ureteral margins: Right ureter, positive for tumor.
6. Distal urethral margin: Negative for tumor.
7. Soft tissue margin or serosa: Positive for tumor (multifocal).

8. Lymph nodes: Soft tissue deposits of urothelial carcinoma at right pelvic lymph node area.

9. pTNM: pT3b,N1,MX, R1, see comment.

Effective this Checklist utilizes the 7th edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

Radical Prostatectomy Pathologic Parameters

1. Gleason Score: 3+3=6.
2. Perineural Invasion: Absent.
3. Tumor Location: Peripheral zone - left (slide B10).
4. Tumor Volume Estimate: 5%.
5. High-grade P.I.N.: Multifocal.
6. Seminal Vesicles: Negative for tumor.
7. Extraprostatic Extension: Absent.
8. Peripheral Margin: Negative for tumor.
9. Distal (apical) Margin: Negative for tumor.
10. Proximal (basilar) Margin: Negative for tumor.
11. Regional Lymph Nodes (right and left): Negative for prostate carcinoma (0/8).
12. pTNM: pT2a,N0,MX.

Effective is Checklist utilizes the 7th edition TNM staging system for prostatectomy of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

Comment

The bladder tumor has a very unusual morphology in that it shows dense solid growth of predominantly epithelioid cells with focal spindling and few loose myxoid areas. The epithelioid tumor cells are high grade, have modest amphophilic cytoplasm and with brisk mitotic activity. In addition, numerous osteoclast-like giant cells are distributed all throughout the tumor. Foci of tumor necrosis are also present.

Immunohistochemical stains performed show that the tumor cells are nuclear positive for GATA3 and with rare nuclear positivity for p63. Keratin Cam5.2, CK7 and CK20 are negative. The positivity for GATA3 and p63 are supportive for urothelial carcinoma. Controls appropriate.

Rare occurrence of osteoclast-like giant cells rich carcinoma, similar to the current case, has been reported in the urinary tract. The few cases previously reported showed an aggressive biologic behavior, see reference.

On part C, the right lymphadenectomy specimen shows large soft tissue deposits

of carcinoma, which may represent overgrowths from a lymph nodal tumor metastasis. No normal lymph node tissue however is identified. Of note, right pelvic lymphadenectomy was performed previously (together with ureterectomy, which was negative for tumor. Nodal status for the current bladder tumor is most consistent with N1.

Reference:

1. Baydar D, Amin MB, Epstein JI. Osteoclast-rich undifferentiated carcinomas of the urinary tract. Mod Pathol. 2006;19:161-71.

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[] MD

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [] MD

Electronically Signed Out by [] MD

Clinical History:

Patient is an year-old male with high-grade urothelial carcinoma undergoing radical cystectomy, prostatectomy, and lymph node dissection.

Specimens Received:

A: Left pelvic lymph node

B: Bladder, prostate, left and right vas deferens

C: Right pelvic lymph node

Gross Description:

The specimens are received in three containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "Left pelvic lymph node". Received in formalin is an aggregate of yellow-tan, soft, and lobulated tissue fragments ranging in size from 1.1-1.4 cm in greatest dimension. The specimen is dissected to reveal 10 lymph node candidates ranging in size from 0.3-1.3 cm in greatest dimension. The specimen is submitted as follows:

A1: 3 lymph node candidates

A2: 2 lymph node candidates

A3: 2 lymph node candidates

A4: 2 lymph node candidates

A5: 1 lymph node candidate

A6: 1 lymph node candidate

B. The second container is additionally identified as, "bladder, prostate, left and right vas deferens". Received fresh and placed in formalin is a 250 g, 12.5 x 7 x 6 cm cystoprostatectomy specimen consisting of a 8.5 x 6.5 x 3 cm bladder with attached mesenteric fat and a detached 3.2 x 2 x 1.7 cm probable tumor (or

prostate). The right seminal vesicles and right vas deferens are not clearly identified and a structure resembling the right seminal vesicles measure 1.5 x 1.5 x 0.4 cm and there is no identifiable right vas deferens structure. The left vas deferens is not clearly identified and the left seminal vesicles measures 2.5x 1.5 x 0.4 cm and a structure resembling the left vas deferens measures 3 x 0.4 cm. The right ureteral stump measures 0.6 x 0.4 cm and demonstrates an intact, patent lumen. The left ureteral stump is absent and the left ureteral resection margin is taken where the ureter enters the bladder and is submitted as B3.

The right half of prostate and bladder is inked blue and the left half is inked black. The bladder and prostate are opened anteriorly along the urethra. The fragments of the outer surface of the tumor fragment are white-brown and shaggy. Sectioning demonstrates a diffuse, homogenous, ill-defined, firm, white-tan.

The opened bladder reveals a 1.8 x 1.2 x 0.3 cm raised, soft, red-pink lesion on the anterior wall designated as lesion #1, a 4 x 1.8 x 0.3 cm raised, soft, red-pink lesion on the inferior posterior wall and right inferior lateral wall designated as lesion #2, a 2.5 x 2.5 cm left posterior wall hemorrhage, and a 1.4 x 1 x 0.2 cm raised, soft, red-pink lesion on the mid left lateral wall designated as lesion #3. There is also an aggregate of firm, white-tan tumor fragments, free floating within the bladder lumen and measuring 11 x 5 x 3 cm. On cut section, lesion #1 extends into the muscularis propria and grossly into the perivesical tissue and is close to the deep margin. On cut section, lesion #2 does not extend beyond the mucosa, and lesion #3 does not extend beyond the mucosa and is 1.0 cm from the deep margin.

There is a 0.6 x 0.5 x 0.5 cm rubbery, white-tan, mass which is in the muscle layer of the left lateral anterior wall and extending into the perivesicular adipose tissue.

The surrounding bladder mucosa is edematous, wrinkled, congested, pink-tan with a uniform 0.1-0.2 cm wall thickness. Bilateral ureteral orifices, adjacent to the trigone, are identified and probe patent.

Representative sections are submitted as follows:

B1: Right ureter resection margin

B2: Distal prostatic urethral margin

B3: Left ureter resection margin (insertion into the bladder

B4-B10: Representative right and left of probable tumor (rule out prostate)

B11-B13 Lesion #1 (closest to deep margin B13)

B14-B16 Lesion #3 (closest to deep margin B16)

B17-B19: Lesion #2

B20: Uninvolved bladder mucosa bladder dome

B21: Uninvolved bladder mucosa anterior wall

B22: Uninvolved bladder mucosa posterior wall

B23: Uninvolved bladder mucosa trigone
 B24: Uninvolved bladder mucosa right lateral wall
 B25: Uninvolved bladder mucosa left lateral wall
 B26: Possible tumor deposit 0.3 cm from deep margin
 B27: Representative vas deferens and seminal vesicles
 B28-B30: Representative free floating tumor fragments
 B31-32: Additional free floating tumor fragments
 B33-B38: Additional possible prostate
 B39-B44: Additional free floating tumor fragments

C. The third container is additionally identified as, "Right pelvic lymph node". Received in formalin is an aggregate of yellow-tan, soft, and lobulated fiber adipose tissue fragments ranging in size from 1-1.5 cm. The specimen is dissected to reveal for lymph node candidates ranging from 0.5-1.5 cm in greatest dimension. The lymph node candidates are submitted as follows:

C1-C3: 1 lymph node candidate, bisected (each cassette)
 C4: 1 lymph node candidate

xxx
 [] MD

See TSS's Discrepancy Form Stating this is urothelial carcinoma.

DQ for histology per BC pathologist.

Criteria	Yes	No
Diagnosis Discrepancy	☒	☐
Primary Tumor Site Discrepancy	☒	☐
HIPAA Discrepancy	☒	☐
Prior Malignancy History	☒	☐
Dup/Synchronous Primary	☒	☐
Case # (circled):	12/20/12	
Reviewed by:	Date Reviewed:	

TCGA Pathologic Diagnosis Discrepancy Form

V4.00

Instructions: The TCGA Pathologic Diagnosis Discrepancy Form should be completed when the pathologic diagnosis documented on the initial pathology report for a case submitted for TCGA is inconsistent with the diagnosis provided on the Case Quality Control Form completed for the submitted case.

Tissue Source Site (TSS): _____ TSS Identifier: _____ TSS Unique Patient Identifier: _____

Completed By (Interviewer Name on OpenClinica): _____ Completed Date: _____

Diagnosis Information

#	Data Element	Entry Alternatives	Working Instructions
1	Pathologic Diagnosis Provided on Initial Pathology Report	<i>Invasive poorly differentiated osteoclast-like giant cells rich urothelial carcinoma</i>	Provide the diagnosis/ histologic subtype(s) documented on the initial pathology report for this case. If the histology for this case is mixed, provide all listed subtypes.
2	Histologic features of the sample provided for TCGA, as reflected on the CQCF.	<i>muscle invasive urothelial carcinoma</i>	Provide the histologic features selected on the TCGA Case Quality Control Form completed for this case.

Discrepancy between Pathology Report and Case Quality Control Form

3	Provide the reason for the discrepancy between the pathology report and the TCGA Case Quality Control Form.	<i>The tumor is all muscle invasive urothelial carcinoma, high grade with morphologically variant features.</i>	Provide a reason describing why the diagnosis on the initial pathology report for this case is not consistent with the diagnosis selected on the TCGA Case Quality Control Form.
4	Name of TSS Reviewing Pathologist or Biorepository Director	_____	Provide the name of the pathologist who reviewed this case for TCGA.

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled.

TSS Reviewing _____ or Biorepository Director

Date _____

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled. The Attending Pathologist or the Department Chairman has been informed or is aware of the above discrepancy in diagnoses.

Principal Investigator Signature _____

Date _____